



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : رضا باشا نظير حسن
المحافظة : القاهرة
الحى / المدينة : مصر القديمة
العنوان : 11 كوم غراب
رقم البطاقة : 27406130102881 رقم التليفون : 01123982775
العمر (تاريخ الميلاد) : 13-06-1974 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان القولون

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان القولون

قسم العلاج :
الأورام

خطة العلاج : أورام كيماوي خط علاج كيميائي ثانى للورم المنتشر طبقا

لبروتوكول (Capecitabine) [14222]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رضا باشا نظير حسن National ID: 27406130102881	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of colon cancer for Capecitabine Chemotherapy Protocol
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية

رضا
 بلشما نظير حسن
 ١١ كوم غرب
 مصر القديمة - القاهرة

١٩٧٤/٠١/٢٣
 ٢٧٤٠٦١٣٠١٠٢٨٨١
 109153934


 ٢٠٢٠/١٠/٢٧
 ربة منزل
 أنثى
 قاسم سيد عبد الحفيظ كحيل
 مسجلة
 متروجة
 البطاقة سارية حتى ٢٠٢٧/١٠/٢٧


 ٢٧٤٠٦١٣٠١٠٢٨٨١



٨٦٦٩

مطلوب
مروءة وحوصلة بالهنيء



ب. هـ




جمهورية مصر العربية
 مبادرة رئيس الجمهورية
 لدعم صحة المرأة المصرية

Entered by: 467 On: 2026-07-07

06/08/2026

MDT FORM

Name: رضا بلتن نظير Hassan	MRN: 8669
Age: 52 Y	ID:
Menopausal Status:	Phone: 01119953620
NRS SCORE: 1	Marital Status:
BMI:	Family History:
CO Morbidity:	Reason Of MDT:

Clinical Data:

BX:

ER PR HER KI67

R

L

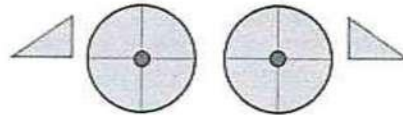
Sonomammography:Mets Workup:History:

metastatic left sided colon (distal half of transverse colon) met to lymph nodes (left supraclav,med ln abdominal ln) liver ,peritoneum .INITIAL PET CT 2/2026 MULTIPLE LEFT SUPRACLAV LN SUV 2.9 MULTIPLE MEDIASTINAL LN M 2.3X2 CM SUV 3.5 FDG AVID MURAL THICKENING OF TRANSVERSE COLON MAX THICKNESS 1.6 SUV 5.7 OMENTAL AND PERITONEAL NODULES SUV 3.2 ABD LN SUV 3.9 BILOBAR HFL LARGEST SEG 5 M 2CM SUV 4.2 PT REC 4 CYCLES XELOX EVALUATION PENDING

hfl biopsy metastatic moderately differentiated

Recommendations:MDT Outcome & Plan

to continue XELOX if good response at evaluation with addition of vectibex



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت					

Patient's Name: Reda Pasha Nazeer
Date: 2026-06-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST,
REVEALED:**

- Bilateral basal bronchiectatic changes forming cystic air spaces. The condition is associated with peri-bronchial thickening and ground glass opacities of both lung fields.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
-

Opinion:

- ❖ Bilateral lower lobar bronchiectatic changes likely post inflammatory sequel.

Dr.Asser Ahmed

General MDT form

Patient Name: Maya Hassan	Age: 52 Y
Sex: Female	Special Habits:
ID: 482/2025	Medical ID: 8669
Family History:	Comorbidity: DM, HTN, SA
Surgical History:	Telephone: 01115953520
Menopausal Status: POST	Marital Status: MARRIED
Referring Physician: MDT	Reason Of MDT: REMDT POST PET-CT
MDT Name: Cancers / GU cancers/ GIT	
Cancers / GU Cancers/ GIT	

Medical Report

Diagnosis: Transverse Colon: Neoplasm, Malignant
Pathology: Picture of intramucosal carcinoma, on top of tubulo-villous adenoma, with extensive high-grade dysplasia, within the limits of the superficially
TTC:
T: 1 N: 1 M: 1
History:
radiology
Investigations:
Recommendations: [11:56 AM, 2/17/2026] Maya Fouad: Diffuse mural thickening and enhancement of the colonic segments as described likely neop
MDT DECISION: For Biopsy from HFL + RAS / BRAF testing + start Xelox meanwhile

Signature	Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatry
	Maya Hassan	Maya Hassan	Maya Hassan			

RAS-BRAF Molecular Report

Name: Reda Basha Nazeer

Age: 51 years.

Clinical data: Metastatic moderately differentiated adenocarcinoma of colonic origin

Specimen type: Tru-cut Biopsy

Date (Received): 24 / 05 / 2026

Date (reported): 07 / 06 / 2026

Lab. Number: 944/26

Referred by:

Indication:

Predictive test for response to therapy.

KRAS	Wild type	
NRAS	Wild type	NA
BRAF	Wild type	NA
		NA

Interpretation & Comment

Based on the current results this patient could be considered for anti-EGFR monoclonal antibody targeted therapy, as the current tumor is expected to achieve clinical response to anti-EGFR monoclonal antibody targeted therapy.

Test Background

Approximately 40% of patients with mCRC harbor an activating KRAS & NRAS mutations in 4% of the patients. The outcome of patients with RAS mutations is worse compared to RAS wild type (RAS wt) cases. However, KRAS & NRAS mutations also have distinct impacts on survival in mCRC; the prognosis of the aggressive NRAS mutant molecular subset is poor. According to National Comprehensive Cancer Network guidelines (NCCN; Version 1.2020) for metastatic colorectal cancer (mCCR), expanded RAS panel for activating mutation of KRAS & NRAS is crucial at time of diagnosis to select the patients who are likely to benefit from anti-EGFR therapies.

Test Description

Tissue sections were reviewed by a pathologist and relevant tumor was selected for analysis. DNA was isolated and amplified by quantitative polymerase chain reaction (qPCR) using NRAS and KRAS CE-IVD kits according to manufacture protocol, results interpreted according to the manufacture user guide. Mutation nomenclature was performed considering the following reference sequences: NM_004985.4 (KRAS) and NM_002524.4 (NRAS gene).



- BRAF V600E mutation specific Competitive allele specific TaqMan assay along with its reference wildtype assay were performed for each sample to accurately interpret the results.

Method and limitation

Method Limitation: The designed primers used in this method and the analysis pipeline are capable of detecting KRAS codons 12, 13, 59, 61, 117 and 146, NRAS codons 12, 13, 59, 61 and 146, and BRAF codon V600E mutations only.

Limit of detection:

- Sensitivity of KRAS-G12D, KRAS-G12A, KRAS-G12V, KRAS-G12S, KRAS-G12R, KRAS-G12C, KRAS-G13D, KRAS-Q61L, KRAS-A146, NRAS-G12D, NRAS-G12S, NRAS-G13D, NRAS-G13R, NRAS-G12C, NRAS-G12V, NRAS-G12A, NRAS-G13V, NRAS-Q61L, NRAS-Q61H and NRAS-K117N mutations are 1% in a background of 99% normal DNA at 10 ng sample DNA amount.
- Sensitivity of KRAS-Q61R, KRAS-Q61H, NRAS-Q61R, NRAS-Q61K & NRAS-A146T mutations are 2% in a background of 99% normal DNA at 10 ng sample DNA amount.

Disclaimer

Molecular Interpretation: Analysis and validity different molecular results and reporting as molecular findings. Clinic pathologic Interpretation: Interpretation and/or recommendation based on the molecular finding in the Clinic pathologic context. This Molecular Genetic test was developed and its performance characteristics determined by the CCHE Molecular Pathology laboratory. This test is used for clinical purposes. This test does not differentiate whether the detected aberration is germline or somatic. Certain sample characteristics may result in reduced sensitivity, including sample heterogeneity, low sample quality, and other causes. The information in this report must be used in conjunction with all relevant clinical information and does not intend to substitute clinical judgement. Decisions on patient care must be based on the independent clinical judgement of the treating physician. A treating physician's decision should not be based solely on this or any other single tests or the information in this report.

References

- (1) Cocco E. et al. Colorectal carcinomas containing hypermethylated MLH1 promoter and wild-type BRAF/KRAS are enriched for targetable kinase fusions. Cancer Res. (2019)
- (2) Lakatos et al. "Current therapy of advanced colorectal cancer according to RAS/RAF mutational status." Cancer and Metastasis Reviews 39.4 (2020).
- (3) Diab, Maria. "Shedding Light on BRAF: Management of Colorectal Cancer in the Era of Personalized Medicine." JCO Oncology Practice (2021).
- (4) Di Nicol Antonio, Federica, et al. "Precision oncology in metastatic colorectal cancer—from biology to medicine." Nature Reviews Clinical Oncology (2021)



(5) Salem, Mohamed E. et al. "Redefining colorectal cancer by tumor biology." American Society of Clinical Oncology Educational Book 40 (2020):

Disclaimer:

- This test reported herein was performed at an external reference lab (57357 laboratory), verified by Biotechnology Molecular Interpretation Clinico-pathologic interpretation

Sarah Sobhy, M.Sc.
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Doaa Hassan, FCB
Pathology Consultant
Marwa Magdy, FCB
Pathology Consultant
Mohamed Tamer
Molecular pathology specialist
Abdel Rahman Mohamed
Molecular pathology specialist

Mohamed Khalifa, M. Sc.
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Printing Date : 08/06/2026 09:23:53 Page 1 of 1 Printing By : marwa mohamed youss

Verified by :
Dr. Duaa Mohamed

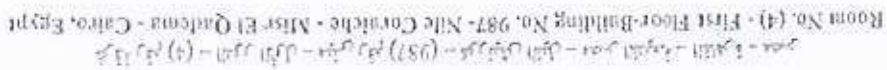
Signature
Dr. Wafaa Ibrahim Ghoneim.
Dr. Duaa Mohamed.

Clinical Data: PET-CT showing metabolically active hepatic, nodal and peritoneal deposits in known case of colonic carcinoma.
Nature of specimen: Hepatic focal lesion, Tru-cut biopsy, collected on 09/04/2026, at 1:40 pm, cold ischemia time: 20 seconds.
Gross examination:
Fragmented grey white tissue cores, ranging in size between 0.2 cm to 1 cm long. Totally submitted.
Microscopic examination:
Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of well formed glands with fused forms and cribriform pattern lined by malignant columnar epithelial cells showing moderate nuclear pleomorphism & hyperchromasia.
Diagnosis:
Hepatic focal lesion, Tru-cut biopsy:
Metastatic moderately differentiated adenocarcinoma, likely of colonic origin.
- Immunohistochemistry is in a separate report.

Pathology Report

Referred by	Dr. Duaa Mohamed
Path No.	944/26
Sex	Female
Age	51 Y
Referred Doc	
Request No.	13005794
Request Date	09/04/2026
Verification Date	14/04/2026
Financial Category	Out Patient
Department	
Registered by	marwa mohamed youss

youss



Page 1 of 1

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Signature
Dr. Waïaa Ibrahim Ghoneim.
Dr. Duaa Mohamed.

Signature

Dr. Duaa Mohamed,

Dr. Walea Ibrahim Ghoneim.

Conclusions:

Hepatic focal lesion, tru-cut biopsy:

- Findings are compatible with metastatic moderately differentiated adenocarcinoma of colonic origin

(Positive CK20 & CDX-2).

Results:

IPT (All controls show appropriate reactivity):

- CK20: Strong positive staining in tumor cells.

• CDX-2: Strong diffuse positive staining in tumor cells.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;

CK20 (SP33) & CDX-2 clone (EPR2764Y).

ischente

ischemia time: 20 seconds.

Nature of specimen: Hepatic focal lesion, Tru-cut biopsy, collected on 09/04/2026, at 1:40 pm, cold

case of colonic carcinoma.

Clinical Data: PET-CT showing metabolically active hepatic, nodal and peritoneal deposits in known

IMMUNOCHEMISTRY REPORT

Path No.	944/26	Sex	Female	Age	51 Y	Referring Doc.	
Patient No.	27406130102881	Request No.	13005794	Request Date	09/04/2026	Verification Date	14/04/2026
		Financial Category	الطبابة - General	Department	Out Patient	IHC No.	
				Registered by	marwa mohamed youssri		

17779

Name: Reda Bashah Naszier

Date: 12/2/2026

ID:30206907

F18-FDG PET/CT STUDY

Clinical History:

- A 51-year-old patient has history of colonic carcinoma.

Reason for PET/CT study:

- Initial Assessment

Technique:

- Radiopharmaceutical: 333MBq of 18F FDG.
- Time from 18F-FDG injection to scan: 41 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.

COMPARISON: No previous PET CT studies.

FINDINGS:

Head and Neck:

- Multiple hyper-metabolic FDG avid left supra and infraclavicular LNDs are noted, the largest measures 1.1x1cm of SUVmax 2.9.
- No FDG avid thyroid nodules
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal diffuse brain activity with NO hypermetabolic brain lesions.

Chest:

- Multiple hypermetabolic retrosternal, posterior mediastinal, subcarinal LNs, the largest measures 2.3x2cm of SUVmax 3.5.
- No axillary lymphadenopathy.
- Left apical subpleural reticulation.
- Bilateral lower lobes cystic bronchiectasis and lingula and middle lobe peribronchial thickening with multiple tiny pulmonary nodules.
- NO FDG avid lung nodules or masses detected.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- FDG avid non-uniform circumferential mural thickening of the distal half of the transverse colon with maximum thickness about 1.6 cm with lumen narrowing and fat smudging, and subsequent dilatation of the proximal part of the transverse colon showing diffuse circumferential mural thickening extends to involve the

ascending colon with fat smudging, reaching thickens about 1.7 cm of SUVmax 5.7.

- Diffuse omental and peritoneal nodular thickening with fat stranding and related thin soft tissue sheets with serosal nodulation, the largest measures about 4.5x1.7cm of SUVmax 3.2.
- Multiple enlarged retro-crural, porto-caval, porta-hepatis, peripancreatic, celiac, mesenteric, aorto-caval, retrocaval and left para-aortic lymphadenopathy, the largest seen at aorto-caval measures 2.7x2cm of SUVmax 3.9. likely metastatic.
- Mild circumferential lower rectal and anal mural thickening with maximum thickness about 1cm.
- The liver is average size with few FDG avid focal lesions, the largest seen at segment V measures 2cm of SUVmax 4.8, likely metastatic.
- The spleen is of average size with NO FDG avid focal lesions detected.
- The pancreas shows normal FDG avidity.
- There are no active adrenal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder.
- Minimal pelvic free fluid.

Musculoskeletal:

- Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

IMPRESSION:

Hypermetabolic active colonic neoplasm.

Metabolically active hepatic, nodal, and peritoneal deposits.

Bilateral sub-centimetric pulmonary nodules, for close follow up.

Thanks for referral,

Noura Adel, MD

Radiology Consultant

Rehab Maklad, MD

Nuclear Medicine Consultant